

PRODUCT LITERATURE

FOLIMIL TABLET 5 MG

Each tablet contains:

Folic Acid 5 mg

Preservative:

Sodium Benzoate 0.1% w/w

Description

Yellow to yellowish light orange round tablet with score on one side and V41 embossed with score on the other side.

Pharmacodynamics

Folic acid is a member of the B group of vitamins, it is necessary for the normal production and maturation of red blood cells. The mucosa of the duodenum and upper part of the jejunum are rich in dihydrofolate reductase, where folates and folic acid are absorbed. Once absorbed, folic acid is rapidly reduced and then methylated to form tetrahydrofolic acid derivatives which are rapidly transported to the tissues.

Pharmacokinetics

Mechanism of action/Effect:

Folic acid, after conversion to tetrahydrofolic acid, is necessary for normal erythropoiesis, synthesis of purine and thymidylates, metabolism of amino acids such as glycine and methionine, and the metabolism of histidine.

Absorption:

Commercially available folic acid is almost completely absorbed from the gastrointestinal tract (mostly in the upper duodenum), even in the presence of malabsorption due to tropical sprue. However, absorption of food folates is impaired in malabsorption syndromes.

Protein binding:

Extensive (to plasma proteins).

Storage

Hepatic (large proportion).

Biotransformation:

Hepatic. Folic acid is converted (in the presence of ascorbic acid) in the liver and plasma to its metabolically active form (tetrahydrofolic acid) by dihydrofolate reductase.

Peak serum concentration:

30 to 60 minutes.

Elimination:

Renal (almost entirely as metabolites). Excess beyond daily needs is excreted, largely unchanged, in urine.

In dialysis, Folic acid is removed by hemodialysis; therefore, dialysis patients should receive increased amounts (100 to 300% of USRDA [United States Recommended Daily Allowances]).

Indication

- For the prevention and treatment of folate deficiency
- For the prevention of neural tube defect in the foetus.

Route of administration

Oral

Recommended Dose

Treatment of folate deficiency

Adult: 10 – 20 mg daily for 14 days or until hematopoietic response obtained.

Daily Maintenance : 2.5 mg – 10 mg

Child (up to 1 year) : 250 mcg / kg daily

(1 – 5 years) : 2.5mg / day

(6 – 12 years) : 5mg / day

Prevention of neural tube defect

5mg daily starting before pregnancy and continued through the first trimester.

Contraindication

Risk-benefit should be considered when the following medical problems exist

»Pernicious anemia

(folic acid will correct hematologic abnormalities but neurologic problems will progress irreversibly; doses of folic acid greater than 0.4 mg per day are not recommended until pernicious anemia has been ruled out, except during pregnancy and lactation)

Sensitivity to folic acid

Warning and Precautions

Pediatrics

Problems in pediatrics have not been documented with intake of normal daily recommended amounts.

Folic acid injection that contains benzyl alcohol as a preservative should not be used in newborn and immature infants. The use of benzyl alcohol in neonates has been associated with a fatal toxic syndrome consisting of metabolic acidosis and CNS, respiratory, circulatory, and renal function impairment.

Geriatrics

Problems in geriatrics have not been documented with intake of normal daily recommended amounts.

Interactions with Other Medicament

Analgesics, long-term use or

Anticonvulsants, hydantoin or

Carbamazepine or

Estrogens or

Oral contraceptives or

Phenobarbital or

Primidone

(requirements for folic acid may be increased in patients receiving these medications)

(concurrent use with folic acid may decrease the effects of hydantoin anticonvulsants by antagonism of their central nervous system [CNS] effects; an increase in hydantoin dosage may be necessary for patients who receive folic acid supplementation)

Antacids, aluminum- or magnesium-containing

(prolonged use of aluminum- and/or magnesium-containing antacids may decrease folic acid absorption by lowering the pH of the small intestine; patients should be advised to take antacids at least 2 hours after folic acid)

Antibiotics

(may interfere with the microbiologic method of assay for serum and erythrocyte folic acid concentrations and cause falsely low results)

Cholestyramine

(concurrent use with folic acid may interfere with absorption of folic acid; folic acid supplementation taken at least 1 hour before or 4 to 6 hours after cholestyramine is recommended in patients receiving cholestyramine for prolonged periods)

Methotrexate or

Pyrimethamine or

Triamterene or

Trimethoprim

(act as folate antagonists by inhibiting dihydrofolate reductase; most significant with high doses and/or prolonged use; leucovorin calcium must be used instead of folic acid in patients receiving these medications)

Sulfonamides, including sulfasalazine

(Inhibit absorption of folate; folic acid requirements may be increased in patients receiving sulfasalazine)

Zinc supplements

(some studies have found that folate may decrease the absorption of zinc, but not in the presence of excessive zinc; other studies have found no inhibition)

Pregnancy and Lactation***Pregnancy:***

Problems in humans have not been documented with intake of normal daily recommended amounts. Folic acid crosses the placenta. However, adequate and well-controlled studies in humans have not shown that folic acid causes adverse effects on the fetus.

Some studies have found that folic acid supplementation alone or in combination with other vitamins given before conception and during early pregnancy may reduce the incidence of neural tube defects in infants.

FDA Pregnancy Category A.

Breast-feeding

Folic acid is distributed into breast milk. However, problems in humans have not been documented with intake of normal daily recommended amounts.

Side Effects***Those indicating need for medical attention******Incidence rare******Allergic reaction, specifically***

bronchospasm (shortness of breath; troubled breathing; tightness of chest; wheezing),

Erythema (reddened skin),

Fever

General malaise (general weakness or discomfort),

Skin rash or itching

Note: Side effects including abdominal bloating and gas, anorexia, confusion, depression, difficulty in concentrating, excitement, irritability, impaired judgment, nausea, trouble in sleeping, and unpleasant taste were reported in a study of patients taking 15 mg daily.

Symptoms and Treatment of Overdose

None

Packing

Packed in blister pack of 3 x 10's, 10 x 10's, 50 x 10's and 100 x 10's

Storage Conditions

Store below 30°C in a dry place, protected from direct light.
Keep away from reach of children.

Expiry Date

2 years from date of manufacture

Name and Address of Product Manufacturer:

TERAPUTICS SDN. BHD. (590500-W)

Lot 10 & 11, PERDA Industrial Park,

Lorong IKS Simpang Ampat B,

14100 Simpang Ampat, S.P.S.,

Pulau Pinang, Malaysia

Name and Address of Product Registration Holder/Distributor:

ZONTRON PHARMACEUTICALS SDN.BHD. (445695-T)

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